

TBI Screening Report - CognitrackX

Patient information

FULL NAME

Redacted Patient

DATE OF BIRTH

01/12/1987

DATE OF INJURY

03/08/2026

DATE OF ASSESSMENT

04/28/2026

Summary of Consciousness and Cognitive Disruption

Based on screening responses, the patient's reported and observed behavior reflects the following

✓ LOC - Loss of consciousness

Endorsed

✓ AOC - Alteration of consciousness

Endorsed

✓ PTA - Post-traumatic amnesia

Endorsed

Overall Classification

🚨 "Likely TBI"

The follow counts represent the patient's self-reported symptoms burden across across three clinical timepoints - Before the injury, within 72 hours of the injury, and currently.

PRE-INJURY BASELINE

4

Symptom before injury

ACUTE (WITHIN 72 HOURS)

14

+10 new after injury

CURRENT / ONGOING

17

+13 above baseline

Understanding TBI Indicators

Clinical criteria for assessing consciousness and cognitive disruption

CLINICAL DEFINITIONS - PER ACRM GUIDELEINES

The following indicators are evaluated using criterion established by the American Congress of Rehabilitation Medicine (ACRM) for the classification of Traumatic Brain Injury. Each criterion reflects a specific disruption to neurological function at the time of injury.

LOC - Loss of Consciousness

A period during which the patient was unresponsive and unaware of their environment following the injury event. Per ACRM criteria, any documented LOC – regardless of duration – is a primary indicator of TBI. LOC lasting up to 30 minutes is consistent with mild TBI; duration exceeding 30 minutes indicates moderate-to-severe classification.

AOC - Alteration of Consciousness

A state of confusion, disorientation, or feeling "dazed" immediately following the injury, without a complete loss of consciousness. ACRM guidelines recognize AOC as a qualifying criterion for mild TBI, as it reflects a transient disruption of normal brain function. Patients may describe feeling "foggy," confused, or unable to think clearly in the immediate aftermath of injury.

PTA - Post-Traumatic Amnesia

A period following the injury during which the patient is unable to form or retain new memories, even if they appear conscious and ambulatory. ACRM defines PTA as a key indicator of TBI severity: PTA lasting less than 24 hours is consistent with mild TBI; 1–7 days indicates moderate TBI; greater than 7 days indicates severe TBI. PTA is frequently underreported as patients often cannot accurately recall this period themselves.

LOSS OF CONSCIOUSNESS

SELF-REPORTED LOSS OF CONSCIOUSNESS

The patient provided strong self-report indicators consistent with a loss of consciousness.

WITNESSED LOSS OF CONSCIOUSNESS

Witness accounts included subtle indicators of possible unconsciousness, such as disorientation or delayed response, but no definitive loss of awareness was observed.

ALTERATION OF CONSCIOUSNESS

SELF-REPORTED ALTERATION OF CONSCIOUSNESS

The patient described substantial alteration in consciousness, including being dazed, mentally blank, or struggling to follow basic instructions. This supports a neurologic disruption consistent with concussion.

WITNESSED ALTERATION OF CONSCIOUSNESS

Witness accounts support a clear alteration of consciousness, including disorganized speech, confusion, or uncharacteristic behavior. These observations align with clinical indicators of mild brain injury.

POS-TRAMATIC AMNESIA / MEMORY

SELF-REPORTED MEMORY LOSS

The patient reported substantial memory loss, including inability to recall key parts of the incident, persistent memory disruption, or retrograde amnesia. These findings are consistent with traumatic brain injury.

WITNESSED MEMORY LOSS

According to the patient witnesses did not report or relate to them issues with memory at or near the time of accident.

Symptom Presentation Timeline

Placeholder symptom chronology matching the PDF's three-band presentation

Pre-Injury Symptoms

4 symptoms

Symptoms reported as present prior to the injury event

HeadachesFatigueMood ImpairmentsExecutive Dysfunction

Symptoms Within 72 Hours of Injury

14 symptoms

Symptoms appeared or worsened within 72 hours, suggesting acute neurological disruption consistent with concussion or mild TBI

HeadachesDizzinessFatigueSensitivity To Light/NoiseSleep DisturbancesBalance ProblemsMemory ProblemsChronic Pain (Associated with Injury Event)Executive DysfunctionImpaired RelationshipsCognitive FatigueSpeech ImpairmentAttention ImpairmentReduced Libido

Persisting, Worsening, or Newly Emerged Symptoms

17 symptoms

Symptoms that have persisted, worsened, or newly developed – suggestive of post-concussive syndrome

HeadachesFatigueSensitivity To Light/NoiseSleep DisturbancesMood ImpairmentsBalance ProblemsMemory ProblemsChronic Pain (Associated with Injury Event)Executive DysfunctionImpaired RelationshipsDisorientationCognitive FatigueSpeech ImpairmentAttention ImpairmentReduced LibidoOccupational Or Educational DifficultyReduced Quality of Life

Summary of Assessment

The patient meets all criteria for a likely traumatic brain injury and continues to report a high burden of persistent, worsening, or newly emerging symptoms. This symptom profile is highly consistent with post-concussive syndrome and may affect cognitive function, emotional regulation, and daily activity. Recommendation: Formal referral for neurocognitive evaluation and multidisciplinary concussion rehabilitation is advised. Coordinated care may include cognitive therapy, vestibular rehabilitation, and behavioral health support depending on symptom complexity.

Clinical Protocol

TBI Treatment Plan Outline

Based on "Likely TBI" classification with 17 persisting or worsening symptoms

🔔 Important

The following recommendations are based on the patient's screening profile and are intended to guide clinical decision-making. All interventions should be reviewed and ordered by a qualified treating provider. Given the Likely TBI classification and ongoing symptom burden, prompt initiation of multidisciplinary care is strongly advised.

📋 STEP 1

Formal Diagnosis & Full Neurocognitive Evaluation

A formal diagnosis is critical at this juncture. The CognitrackX screening provides an initial indicator of neurocognitive impact, but a comprehensive evaluation is required to establish diagnosis, severity, and a personalized treatment pathway.

REQUIRED EVALUATIONS

✓ Formal TBI Diagnosis

By a board-certified neurologist or neuropsychologist with documented TBI specialization.

✓ Full Neurocognitive Evaluation

Comprehensive assessment of memory, attention, processing speed, and executive function using validated instruments.

✓ Neurobehavioral Scoring & Assessment

Evaluating emotional regulation, impulse control, and behavioral changes post-injury through standardized behavioral instruments.

✓ Baseline Functional Assessment

Documents pre-treatment functional status to enable accurate ongoing comparison and treatment progress tracking.

NEUROIMAGING – WHEN WARRANTED

Depending on the findings of the formal diagnosis and full evaluation, advanced neuroimaging may be indicated:

✓ MRI-DTI (Diffusion Tensor Imaging)

The gold standard for identifying white matter tract damage not visible on standard MRI – highly recommended for moderate-to-severe presentations.

✓ Standard MRI or CT

Indicated where acute structural findings such as hemorrhage or contusion are suspected.

✓ Functional Neuroimaging (fMRI or SPECT)

For complex or chronic presentations where structural imaging may be insufficient to capture functional deficits.

Documentation standard:

Providers selected for evaluation should fully document and report all data findings using quantitative, validated scoring instruments. Thorough clinical documentation is essential for both treatment continuity and any associated legal proceedings.

🧠 STEP 2

Treatment Plan & Therapeutic Interventions

Based on Valhalla Health's clinical experience across more than 50,000 TBI treatments, the following evidence-informed and documented protocols have demonstrated meaningful efficacy in TBI recovery. Treatment plans should be individualized to the patient's specific diagnosis, symptom profile, and functional goals.

CELLULAR & NEUROMODULATION PROTOCOLS

✓ Photobiomodulation (PBM) Therapy

Documented light-based protocols targeting neuroinflammation and mitochondrial function.

✓ Neuromodulation Protocols

Including tDCS and related modalities targeting neural circuit regulation and recovery.

✓ Nebulized Exosome Therapy

Emerging regenerative protocol demonstrating efficacy in neuroinflammation and recovery support.

PHYSICAL & SENSORIMOTOR REHABILITATION

✓ Vestibular Rehabilitation

Critical for patients presenting with dizziness, balance deficits, or post-concussive vestibular dysfunction – a highly common TBI sequela.

✓ Neuro-Ocular Motor Rehabilitation

Addressing vision processing disruptions, convergence insufficiency, and saccadic dysfunction – common following TBI and frequently underdiagnosed.

✓ Physical Therapy (TBI-Specific Protocols)

Targeting fatigue management, coordination deficits, and functional mobility with protocols adapted for post-TBI presentations.

COGNITIVE & BEHAVIORAL THERAPIES

The following psychotherapeutic modalities have shown particular efficacy in TBI populations:

✓ Cognitive Behavioral Therapy (CBT)

Specifically adapted protocols for post-injury cognitive and emotional dysregulation – well-supported by TBI outcome literature.

✓ EMDR (Eye Movement Desensitization and Reprocessing)

Particularly effective where trauma and TBI co-occur, which is common in motor vehicle accident and assault populations.

✓ ART (Accelerated Resolution Therapy)

A rapid, evidence-based trauma therapy with growing TBI application data and strong patient-reported outcomes.

✓ Cognitive Rehabilitation Therapy (CRT)

Structured, systematic retraining of memory, attention, and executive function through individualized cognitive exercises.

🧑‍🤝🧑 STEP 3

Comorbid Psychological Assessment

TBI frequently co-occurs with significant psychological sequelae that require independent evaluation and treatment. Failure to identify and address these conditions can substantially impede recovery and functional outcomes.

ASSESSMENTS WARRANTED

✓ Depression Screening

Using validated instruments (PHQ-9, BDI-II) given the high prevalence of post-TBI depressive disorder across all severity levels.

✓ Anxiety Evaluation

Generalized anxiety and panic presentations are common post-injury and require targeted intervention – often undertreated in TBI populations.

✓ PTSD Assessment

Particularly where the injury occurred in the context of a traumatic event. Use of PCL-5 or clinician-administered CAPS-5 is recommended.

✓ Sleep Disorder Screening

Disrupted sleep architecture is both a symptom and a driver of TBI recovery impairment – identifying and treating sleep disorders significantly improves overall outcomes.

These assessments should be conducted by licensed mental health professionals with documented TBI experience. Providers must use standardized, validated instruments and report quantitative scoring for all evaluations – narrative-only reports are insufficient.

👤 STEP 4

Provider Selection & Documentation Standards

The quality and completeness of clinical documentation directly impacts both patient outcomes and the strength of any associated legal case. Valhalla Health strongly recommends the following standards when selecting providers across all steps of this plan:

✓ Structured, Standardized Documentation

Select providers who utilize structured documentation protocols for all evaluations and treatment sessions, not narrative-only notes.

✓ Quantitative Scoring on Validated Instruments

Ensure all providers generate numerical scores on validated tools – these are critical for establishing severity, tracking progress, and supporting expert testimony.

✓ Regular Progress Documentation

Require documentation at defined intervals throughout the treatment course to demonstrate treatment response and functional change over time.

✓ Legally Defensible Reporting Capability

Confirm that providers can produce comprehensive, structured reports suitable for expert review and use in legal proceedings.

✓ Documented TBI Clinical Experience

Where possible, engage providers with specific TBI clinical experience and verifiable patient outcome data, general practitioners are insufficient for complex TBI cases.

Valhalla Health's Neuro Sentinel network can connect your firm with credentialed providers meeting these documentation standards nationwide. Contact your Valhalla Health representative for a provider referral in your region.

🚨 Immediate Action Recommended

Research data is unambiguous: **early intervention following TBI leads to the best patient outcomes.** Neuroplasticity – the brain's capacity for repair and reorganization – is most active in the acute and subacute phases following injury. Delays in diagnosis and treatment allow secondary injury cascades to progress and reduce the window for maximum recovery. We strongly recommend immediate action toward all steps outlined in this plan. The Valhalla Health clinical team is available to support coordination of care, provider referrals, and clinical consultation throughout this process.

DISCLAIMER

This document is intended to support clinical recommendations and neurocognitive screening by offering critical analysis of neuropsychological evaluations and relevant scientific literature. It may help inform decisions about further assessment, referral, or care planning. This document is not intended to serve as a formal medical or psychological diagnosis. Diagnostic conclusions must be made by a qualified provider following direct evaluation and integration of comprehensive clinical data.

VALHALLA HEALTH

📅 04/23/2026

📞 Valhalla clinical sample

🔒 Redacted preview only